

Dermatological Management

The benefit/risk ratio and the need for treatment must be rigorously discussed and justified. In newborns, a local antiseptic may be considered to prevent potential infection. Laser treatment of hyperpigmented lesions should be avoided, as it has been associated with recurrent inflammatory episodes. Photoprotection is recommended due to the risk of cutaneous inflammation and abnormal pigmentation.

Verrucous lesions located under the nail that are painful require diagnosis via skin biopsy and appropriate treatment. The standard approach involves surgical excision with curettage of the underlying bone; repeated surgeries may be necessary. Complete regression has been reported in one case following a 6-month course of oral etretinate (a retinoid). Topical retinoic acid (0.05%) has also shown efficacy, with resolution of pain and verrucous tumors observed within 1 to 6 months in one patient.

Dermatological follow-up should be scheduled monthly during the first 6 months of life, then every trimester during the first year, and annually until the age of 5. Beyond this age, monitoring should be adapted based on disease progression. The frequency of visits should be increased in cases of prolonged, extensive inflammatory lesions or disabling verrucous wounds, especially during the first year of life.

Ophthalmological Involvement and Follow-up

Ophthalmological evaluation should begin promptly after clinical diagnosis of incontinentia pigmenti (IP). Priority must be given to clinical examination of the peripheral retina, performed under complete pupillary dilatation using the protocol established for retinopathy of prematurity screening. This allows

indirect ophthalmoscopy by a physician experienced in neonatal retinal examination.

In cases where peripheral vasculopathy is detected, examination under general anesthesia is recommended. When feasible, this should include retinal photography and fluorescein angiography, with consideration for argon laser photocoagulation if indicated.

Follow-up:

- **After early laser treatment:** Clinical examinations should be performed at 15, 30, and 45 days, and at 2 and 3 months post-treatment.
- Ongoing monitoring should be tailored according to initial findings and disease activity.

Table 3: Summary Card for the Therapeutic Management of IP Patients

Specialty	Management Recommendations
Dermatology	- Close monitoring in the first months of life ● Monthly during the first 6 months ● Every trimester during the first year ● Annually until age 5 ● Then individualized based on disease progression - One annual visit at a reference center with multidisciplinary assessment if needed, continuing into adulthood - Increase visit frequency in cases of prolonged, extensive inflammatory lesions or disabling verrucous lesions
Ophthalmology	- At time of IP diagnosis: ● Perform clinical examination of the peripheral retina with full pupillary dilatation ● If peripheral vasculopathy is present, proceed to examination under general anesthesia (with retinal imaging and fluorescein angiography when possible) and consider argon laser treatment - Follow-up: ● After early laser treatment: clinical exams at day 15, 30, 45, and at month 2 and 3 post-treatment ● Further follow-up based on initial findings and treatment response